

Dr. Malcolm Still

Rheumatologist

5 Grant St

Fairwood

03/05/2014

Dear Dr. Still,

Re: Mr James Seymour

DOB: 19/09/1953

I am writing to refer Mr Seymour, a 60-year-old retired academic, whose symptoms and signs are suggestive of gout. Your expert assessment and further management would be highly appreciated.

Mr Seymour, who is a heavy alcohol consumer, has a family history of rheumatoid arthritis related to his father. He has had gout, with recurrent attacks of exacerbations for which he has been receiving Colchicine, Indomethacin and allopurinol.

On 25/04/2014, Mr Seymour attended my clinic complaining of an acute episode of gout. Despite he tried to control it by an adjustment to his medication and modifying the diet, this exacerbation was severe enough to wake him from sleep. A moderately inflamed left big toe was detected upon his examination. He was irritable and being in pain, as well as insistence on the possibility of rheumatoid arthritis. His treatment doses were modified, and Oxycodone, 5mg at bed time was prescribed. He was advised to stick to a healthier life style.

One week later, the previously requested investigations revealed minor degenerative changes of the left first metatarsophalangeal joint. High levels of urate, CRP and MCH were found. His condition demonstrate enhancement. Consequently, he was counselled on a continuation of allopurinol and adherence to the life style adjustment. A discussion, regarding a synovial fluid sample was conducted.

In view of above, kindly contact Mr Seymour on the pathology result.

Should you have any queries, please do not hesitate to contact me.

Yours sincerely

Doctor

